

Dear Mr. Su Tieshan,

Thank you for visiting iKang Junan (Shanghai iKang Junan Outpatient Clinic Co., Ltd.) and for your trust and support. Please find enclosed your medical examination report for 12 June 2025.

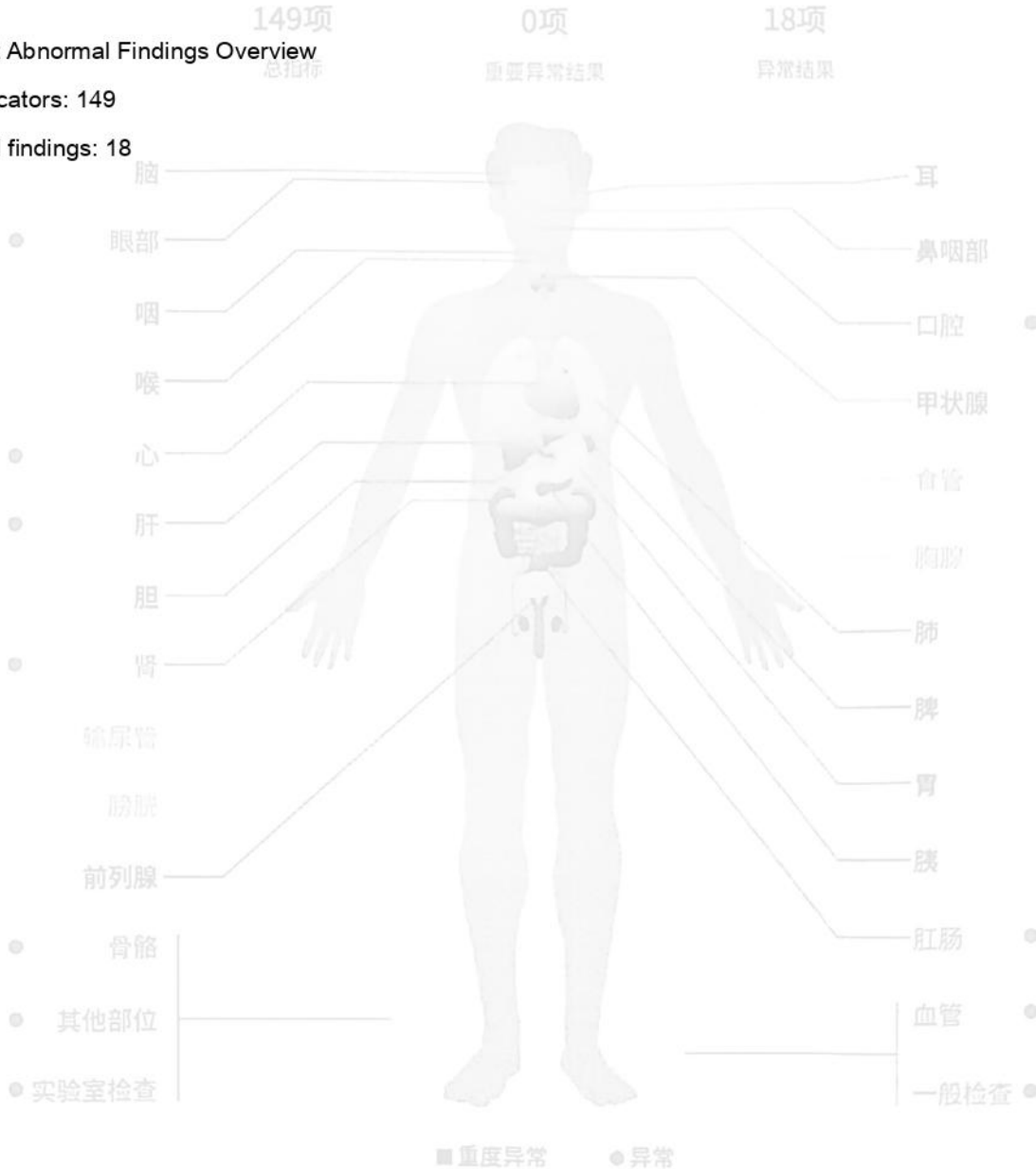
Guide to Reading This Report

1. This report consists of health information, major findings from this examination, relevant conditions, and recommendations.
2. The examination data in this report reflect only the results of the tested items and organs covered in this visit; they do not represent all organs or all possible health indicators of the human body.
3. The conclusions in this report are based on the health information you provided and the clinical findings obtained during this examination. Concealed or inaccurate information may mislead the physician and affect interpretation. Incomplete health information may also lead to deviations in the conclusions.
4. Because different examination methods are used, results concerning the same organ or system may differ.

Important Abnormal Findings Overview

Total indicators: 149

Abnormal findings: 18



Original page image retained in background. Translation provided for reference. 晋唐君安 外事体检报告 苏华山 牛牛 用户ID 2075889022 体检号 138750632056

1. 体检重要异常结果、复查建议及治疗建议

阳性结果和异常情况

English Translation - Page 3

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2. Specialist Recommendations and Guidance

2. 专家建议与指导

01. Decreased f-PSA / t-PSA ratio

1. Prostate-specific antigen (PSA) is a protein secreted mainly by prostatic epithelial cells.
2. When total PSA is 4-10 ng/mL, the f-PSA/t-PSA ratio may assist in the differential diagnosis between prostate cancer and benign prostatic hyperplasia. If both total PSA and free PSA are elevated and the f-PSA/t-PSA ratio is decreased (<10%), the possibility of prostate cancer should be considered, and prostate biopsy may be required for definitive diagnosis.
3. Further comprehensive evaluation and management in hospital are recommended. 须进行前列腺穿刺活检来明确诊断。

02. Obesity; increased waist circumference

1. Body mass index (BMI = weight [kg] / height [m]²) ≥ 28 indicates obesity. A waist circumference ≥ 85 cm in men or ≥ 80 cm in women indicates central obesity.
2. Waist circumference is the simplest and most practical indicator for assessing abdominal fat accumulation (central obesity).
3. Obesity results from imbalance between energy intake and expenditure and is often associated with genetics, endocrine factors, lifestyle, exercise habits, and psychological factors. Obesity can lead to hypertension, cardiovascular disease, diabetes, fatty liver disease, malignancy, and many other disorders.
4. It is recommended that you improve your lifestyle by maintaining balanced nutrition and a rational diet (low energy, low fat, appropriate high-quality protein, appropriate cereals, and adequate fresh vegetables and fruits), stopping smoking, limiting alcohol, exercising effectively, and maintaining psychological balance.
5. Increase exercise to consume excess fat. Select a suitable type and duration of exercise according to your age and physical condition to achieve weight reduction.

03. History of hypertension; blood pressure normal at this examination

1. Hypertension is a chronic multifactorial disease. Blood pressure should be monitored carefully, and standardized antihypertensive therapy should be continued under the guidance of a cardiologist so that blood pressure remains within the normal range. Regular follow-up is necessary; do not discontinue medication on your own.
2. Correct unhealthy lifestyle habits such as prolonged sitting, insufficient exercise, excessive stress, and irritability. Balance work and rest, maintain a good mindset, and participate in appropriate physical activity while keeping body weight within a normal range.
3. Follow a low-salt, low-sugar, low-fat diet; stop smoking; limit alcohol; and avoid strong tea and coffee.
4. Long-term uncontrolled hypertension may cause damage to major organs and serious complications, such as cardiac enlargement, myocardial injury, cerebral hemorrhage, myocardial infarction, and retinal arteriosclerosis. Therefore, regular hospital examinations of the heart, brain, kidneys, and fundus are recommended. If blood pressure rises abruptly and is accompanied by headache, vomiting, palpitations, or chest tightness, seek medical attention promptly.
5. Blood pressure was normal in this examination; cardiology follow-up is recommended.

04. Sinus bradycardia

1. This refers to a heart rate < 60 beats/min. In most cases, bradycardia has no major clinical significance; for example, it may be seen in athletes or individuals who exercise regularly. In a minority of cases it may be associated with coronary heart disease or sick sinus syndrome.
2. If you experience chest tightness, palpitations, dyspnea, or marked bradycardia, 24-hour ambulatory electrocardiographic monitoring may be required. Please attend the Department of Cardiology for further evaluation and management.

05. Prominent aortic arch

1. This is commonly seen in longstanding inadequately controlled hypertension leading to aortic change and cardiac enlargement, and may also be seen in obesity.



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06. Elevated indirect bilirubin

1. Hepatobiliary disease may cause an increase in bilirubin.
2. Bilirubin test results are easily influenced by various metabolic conditions.
3. Mild elevation usually has no major clinical significance.
4. Repeat testing is recommended. If the level remains clearly elevated on repeat testing, specialist evaluation is advised.

07. Decreased plateletcrit

Clinical correlation is recommended; repeat testing may be considered if necessary.

08. Mild fatty liver

1. This indicates mild accumulation of fat within hepatocytes beyond the physiological amount.
2. Strengthen physical exercise, such as running, walking, aerobics, or tai chi, to promote lipid metabolism.
3. Periodic re-examination is recommended. If liver function becomes abnormal, please seek evaluation and treatment in gastroenterology / hepatology.

09. Bilateral renal crystals

Renal crystals may be solitary or multiple. Crystal-related obstruction may be accompanied by hydronephrosis. Small crystals that do not cause symptoms may not require treatment. Drink plenty of water in daily life, especially replenish fluids promptly after exercise, so as to prevent concentrated urine from forming renal calculi or enlarging existing crystals. If crystals are large or if renal colic and/or hydronephrosis occurs, please seek treatment in urology.

10. Cervical osteophyte formation / cervical degenerative change

1. Cervical hyperostosis is related to chronic strain, aging, and degenerative changes.
2. Management should depend on the severity of the degenerative change.
3. If you experience headache, dizziness, or numbness of the upper limbs, please attend a specialist clinic for evaluation and treatment.

12 阑尾手术史

11. Status post anal fistula surgery

If any discomfort is present, specialist follow-up is recommended.

12. History of appendectomy

If any discomfort occurs, please return to hospital for specialist review.

13. Refractive error

Pay attention to eye hygiene and undergo regular ophthalmologic follow-up.

14. Increased cup-to-disc ratio in both eyes

Please attend ophthalmology for further examination of intraocular pressure and visual field testing, to exclude glaucoma.

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3. Medical Examination Results

General Examination

Height: 179.0 cm

Weight: 91.5 kg

Body Mass Index (BMI): 28.6 (reference 18.5-23.99)

Systolic Blood Pressure: 129 mmHg (reference 90.0-139.0)

Diastolic Blood Pressure: 84 mmHg (reference 60.0-89.0)

Waist Circumference: 100 cm (reference 0-84.99)

Interpretation: 1. Obesity 2. Increased waist circumference

Internal Medicine

History: 5-year history of hypertension, under treatment

Past history: No special history

Pulse: 66 beats/min

Heart rhythm: Regular

Cardiac examination: Normal

Lung auscultation: No abnormal breath sounds heard bilaterally

Liver: Not palpable below the costal margin

Kidney region: No tenderness over either renal area

Other internal medicine findings: None

Surgery / Physical Examination

(continued on next page)

心率

66

次/分

心律

齐

心音

正常

肺部听诊

双侧呼吸音未闻及异常

肝脏触诊

肝脏肋下未触及

脾脏触诊

脾脏肋下未触及

肾脏叩诊

双肾区无叩痛

内科其它

无

初步意见

高血压病史

外科

检查者: 沈维君



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Surgical / Physical Examination

检查项目	检查所见	单位
Skin: No obvious abnormality detected		
Superficial lymph nodes: No obvious abnormality detected		
Head and neck: No obvious abnormality detected		
Chest / spine / extremities: No obvious abnormality detected		
Anorectal / genital examination: Examination declined		
Other surgical findings: No obvious abnormality detected		

Ophthalmology

Uncorrected visual acuity (right): 0.8	未见明显异常	
Uncorrected visual acuity (left): 0.8	放弃检查	
Color vision: Normal		
External eye: No obvious abnormality detected	放弃检查	
Conjunctiva / cornea / lens: No obvious abnormality detected	放弃检查	
Fundus: Enlarged physiologic optic cup in both eyes; cup-to-disc ratio >0.5		
Non-contact intraocular pressure, right eye: 14 mmHg	阑尾术后、肛瘘术后	
Non-contact intraocular pressure, left eye: 15 mmHg	阑尾术后、肛瘘术后	

眼科

检查者: 李军、薛中林

检查项目	检查所见	单位
裸视力(右)		
裸视力(左)		
矫正视力(右)	0.8	
矫正视力(左)	0.8	
色觉	正常	
外眼	未见明显异常	
眼科其它	无	
裂隙灯检查	未见明显异常	
眼底镜检查	双眼视乳头凹陷扩大, 杯/盘比大于0.5	
右眼非接触性眼压	14	mmHg
左眼非接触性眼压	15	mmHg

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Fundus Photography

Both eyes: enlarged physiologic optic cup; cup-to-disc ratio >0.5. Retinal vessels and macula are normal.

Impression: 1. Increased cup-to-disc ratio in both eyes 2. Refractive error

Otorhinolaryngology (ENT)

Past history: No special history

External ear: No obvious abnormality detected

External auditory canal: No obvious abnormality detected

Tympanic membranes: No obvious abnormality detected

Nasal septum: No obvious abnormality detected

Nasal cavity / pharynx / tonsils: No obvious abnormality detected

Other ENT findings: None

Hearing (128 Hz tuning fork): Normal

Complete Blood Count (part 1)

WBC 7.84 (3.5-9.5) x10^9/L

RBC 5.10 (4.3-5.8) x10^12/L

咽	未见明显异常
扁桃体	未见明显异常
耳鼻咽喉科其它	无
听力(128HZ音叉)	正常

初步意见 未见明显异常

血常规

操作者: 杜秋香 审核者: 侯大年

检查项目	缩写	测量结果	提示	参考区间	单位	检验单位	样本类型	方法学
白细胞计数	WBC	7.84		3.5—9.5	10^9/L	爱康君安健疗国际上海旗舰店	血	
红细胞计数	RBC	5.10		4.3—5.8	10^12/L	爱康君安健疗国际上海旗舰店	血	

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Mean Platelet Volume (MPV): 10.7 (7.6-13.2) fL

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爱康君安 健康体检报告 MEDICAL EXAMINATION REPORT 苏铁山 先生 用户ID 2075889023 体检号 1382506120011

Complete Blood Count (part 3)

Platelet Distribution Width (PDW): 12.6 (9.8-16.1) fL

Lymphocyte Percentage (LYMPH%): 34.80 (20-50) %

Neutrophil Percentage (NEUT%): 56.6 (40-75) %

Absolute Lymphocyte Count (LYMPH): 2.73 (1.1-3.2) x10^9/L

Absolute Neutrophil Count (NEUT): 4.44 (1.8-6.3) x10^9/L

Red Cell Distribution Width - SD (RDW-SD): 41.9 (35-54) fL

Plateletcrit (PCT): 0.16 (0.17-0.35) % - low

Large Platelet Ratio (P-LCR): 30.6 (13.0-43.0) %

中性粒细胞百分比

NEUT%

56.6

40—75

%

淋巴细胞绝对值

LYMPH

2.73

1.1—3.2

10^9/
L

中性粒细胞绝对值

NEUT

4.44

1.8—6.3

10^9/
L

红细胞分布宽度-标准差

RDW-SD

41.9

35—54

fL

血小板压积

PCT

0.16

↓

0.17—0.35

%

大血小板比率

P-LCR

30.6

13.0—43.0

%

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Urinalysis (part 1)

Specific Gravity (SG): 1.030

Urine pH: 6.0 (4.5-8.0)

Leukocytes (LEU): Negative

Nitrite (NIT): Negative

Protein (PRO): Negative

Glucose (GLU): Negative

Ketone (KET): Negative

Urobilinogen (URO): Negative

尿比重	SG	1.030	1.003—1.030	爱康君安健疗国际上海旗舰店中心	尿液
尿白细胞	LEU	阴性	阴性	爱康君安健疗国际上海旗舰店中心	尿液
尿亚硝酸盐	NIT	阴性	阴性	爱康君安健疗国际上海旗舰店中心	尿液
尿蛋白质	PRO	阴性	阴性	爱康君安健疗国际上海旗舰店中心	尿液
尿糖	GLU	阴性	阴性	爱康君安健疗国际上海旗舰店中心	尿液
尿酮体	KET	阴性	阴性	爱康君安健疗国际上海旗舰店中心	尿液
尿胆原	URO	阴性	阴性	爱康君安健疗国际上海旗舰店中心	尿液

Urinalysis (part 2)

Bilirubin (BIL): Negative

Occult Blood (BLD): Negative

Urine Microscopy - RBC: 0 (0-3) /HP

Urine Microscopy - WBC: 0 (0-5) /HP

Casts (CAST): 0 (0) /LP

Clarity: Clear

Color: Light yellow

Other findings: None

	BIL	阴性	阴性		爱康君安健疗国际上海旗舰店中心	尿液	
尿胆红素	BLD	阴性	阴性		爱康君安健疗国际上海旗舰店中心	尿液	
尿胆红素	RBC	0	0—3	/HP	爱康君安健疗国际上海旗舰店中心	尿液	
尿胆红素	WBC	0	0—5	/HP	爱康君安健疗国际上海旗舰店中心	尿液	
尿胆红素	CAST	0	0	个/LP	爱康君安健疗国际上海旗舰店中心	尿液	
尿胆红素	UCI	清	清		爱康君安健疗国际上海旗舰店中心	尿液	
尿胆红素	Colour	淡黄	淡黄		爱康君安健疗国际上海旗舰店中心	尿液	
尿胆红素		未见	无		爱康君安健疗国际上海旗舰店中心	尿液	

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Urine Microalbumin and Laboratory Tests

Urine microalbumin (U-ALB): 7.2 (0-30) mg/L

注：本结果只对此条码来样负责，仅供临床参考。

Tumor Markers / Laboratory Panel

Alpha-fetoprotein (AFP): 2.41 (0-7) ng/mL

尿微量白蛋白定量 操作者：李文杰 审核者：张林彬

检测项目	缩写	测量结果	提示	参考区间	单位	检验单位	样本类型	方法学
尿微量白蛋白定量	U-ALB	7.2		0—30	mg/L	上海金城医学 检验所有限公司	尿液	

小结 未见明显异常

注：本结果只对此条码来样负责，仅供临床参考。

呼气实验室

操作者：黄宏喜 审核者：侯大年

检测项目	缩写	测量结果	提示	参考区间	单位	检验单位	样本类型	方法学
幽门螺杆菌检测 (C13 呼气试验)	C13	2		0—4		爱康君 安健疗 国际上海 海旗舰中心	气	

小结 未见明显异常

注：本结果只对此条码来样负责，仅供临床参考。

实验室检查

操作者：金晓 罗鑫鑫 王洁 李丹丹 审核者：侯大年 邵琰婕 罗雅文 陆芳梅

检测项目	缩写	测量结果	提示	参考区间	单位	检验单位	样本类型	方法学
甲胎蛋白 (T-12)	AFP	2.41		0—7	ng/ml	爱康君 安健疗 国际上海 海旗舰中心	血	

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Laboratory Panel (continued)

Carcinoembryonic Antigen (CEA): 1.78 (0.00-5.00) ng/mL

Alanine Aminotransferase (ALT): 21 (0-50) U/L

Aspartate Aminotransferase (AST): 24 (0-40) U/L

Gamma-Glutamyl Transferase (GGT): 15 (5-61) U/L

Alkaline Phosphatase (ALP): 67 (42-141) U/L

Total Bilirubin (TBIL): 20.17 (0-21) umol/L

Direct Bilirubin (DBIL): 5.82 (0-7) umol/L

Indirect Bilirubin (IBIL): 14.35 (0-14) umol/L - mildly elevated

癌胚抗原 (CEA)	CEA	1.78	0.00—5.00	ng/ml	爱康君安健疗国际上海旗舰中心	血
天门冬氨酸氨基转移酶	AST	24	0—40	U/L	爱康君安健疗国际上海旗舰中心	血
γ-谷氨酰转移酶	GGT	15	5—61	U/L	爱康君安健疗国际上海旗舰中心	血
碱性磷酸酶	ALP	67	42—141	U/L	爱康君安健疗国际上海旗舰中心	血
总胆红素	TBIL	20.17	0—21	umol/L	爱康君安健疗国际上海旗舰中心	血
直接胆红素	DBIL	5.82	0—7	umol/L	爱康君安健疗国际上海旗舰中心	血
间接胆红素	IBIL	14.35	↑ 0—14	umol/L	爱康君安健疗国际上海旗舰中心	血

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Laboratory Panel (continued)

Total Protein (TP): 74.1 (60-85) g/L

Albumin (ALB): 44.7 (35-55) g/L

Globulin (GLB): 29.40 (15-45) g/L

Albumin/Globulin Ratio (A/G): 1.5 (1.0-2.5)

Total Bile Acid (TBA): 5 (0-10) umol/L

Cholinesterase (CHE): 8201 U/L

Urea (UREA): 5.12 (2.76-8.07) mmol/L

Creatinine (Cr): 92.00 (59-104) umol/L

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Laboratory Panel (continued)

Uric Acid (UA): 332.00 $\mu\text{mol/L}$

Fasting Blood Glucose (FBG): 4.36 (3.5-6.1) mmol/L

Total Cholesterol (TC): 5.01 (3.1-5.7) mmol/L

Triglycerides (TG): 1.03 (0.45-1.70) mmol/L

High-Density Lipoprotein Cholesterol (HDL-C): 1.55 (0.8-1.8) mmol/L

Low-Density Lipoprotein Cholesterol (LDL-C): 3.44 (2.07-3.6) mmol/L

High-sensitivity C-reactive protein (hs-CRP): 0.60 (0-3) mg/L

Serum Iron: 10.19

Calcium (Ca): 2.31 (2.08-2.60) mmol/L

总胆固醇

TC

5.01

3.1—5.7

mmol/L

血

甘油三酯

TG

1.03

0.45—1.70

mmol/L

血

高密度脂蛋白胆固醇

HDL-C

1.55

0.8—1.8

mmol/L

血

低密度脂蛋白胆固醇

LDL-C

3.44

2.07—3.6

mmol/L

血

超敏C反应蛋白

HS-CRP

0.60

0—3

mg/L

血

同型半胱氨酸

Hcy

10.19

0—15

$\mu\text{mol/L}$

血

血清钙

Ca

2.31

2.08—2.60

mmol/L

血

★
车建高

<6岁

胡李长

李叶子

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Laboratory Panel (continued)

Phosphorus (P): 1.05 (0.85-1.51) mmol/L					上海金域医学检验所有限公司	血
Magnesium (Mg): 1.00 (0.75-1.02) mmol/L	血清镁	1.05	0.85—1.51	mmol/L		
Total PSA (tPSA): 1.560 (0-4.0) ng/mL						
Free PSA (fPSA): 0.327 (0-0.9) ng/mL						
f-PSA / t-PSA Ratio: 0.21 (0.25-1) - decreased						
CA19-9: 12.92 U/mL					上海金域医学检验所有限公司	血
Neuron-Specific Enolase (NSE): 9.51 ng/mL	血清NSE	1.00	0.75—1.02	mmol/L		
CYFRA21-1: 2.00 (0.00-3.30) ng/mL						
总前列腺特异性抗原 (T-12)	T-PSA	1.560	0.000—4.000	ng/ml	爱康君安健疗国际上海旗舰店中心	血
游离前列腺特异性抗原 (T-12)	f-PSA	0.327	0—0.9	ng/ml	爱康君安健疗国际上海旗舰店中心	血
F-PSA/T-PSA	F-PSA/T-PSA	0.21	↓ 0.25—1		爱康君安健疗国际上海旗舰店中心	血
糖类抗原19-9 (T-12)	CA19-9	12.92	0.00—27.00	u/ml	爱康君安健疗国际上海旗舰店中心	血
神经元特异性烯醇化酶 (T-12)	NSE	9.51	0.00—16.30	ng/ml	爱康君安健疗国际上海旗舰店中心	血
细胞角蛋白19片段 (T-12)	CYFRA21-1	2.00	0.00—3.30	ng/ml	爱康君安健疗国际上海旗舰店中心	血

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Additional Laboratory Tests and Ultrasound Summary

Free Triiodothyronine (FT3): 1.62 (0.92-2.79) nmol/L
Sodium (Na): 138.40 mmol/L
Thyroid Stimulating Hormone (TSH): 2.48 (0.55-4.78) mIU/L

Electrocardiogram (ECG)

Impression: Sinus bradycardia; heart rate 57 beats/min.

Abdominal Ultrasound

Liver: Increased echogenicity of the hepatic parenchyma, compatible with fatty infiltration.
Gallbladder / pancreas / spleen: No obvious abnormality detected.
Spleen: Normal morphology and size; capsule intact and smooth; internal echo distribution homogeneous; splenic hilar vessels normal.
Kidneys: Punctate hyperechoic foci with comet-tail / star-like echoes seen within the kidneys; intrarenal blood flow normal on CDFI.

小结

1. F-PSA/T-PSA比值降低
2. 间接胆红素增高

爱康君安体检中心温馨提示：仅供参考。

心电图室

检查者： 刘毛一

检查项目	检查结果	单位
心电图	窦性心律，心率<60次/分；心率57次/分	

初步意见

窦性心动过缓

腹部彩超

检查者： 陈三

检查项目	检查结果	单位
肝	肝脏形态大小正常，肝内实质回声细腻，分布欠均匀，血管纹理走行清晰，门静脉正常。CDFI:血流显示正常	
胆	胆囊形态大小正常，囊壁光整，胆汁充盈良好，胆总管内径正常	
胰	胰腺形态大小正常，内部回声分布均匀，主胰管未见扩张。CDFI:血流显示正常	
脾	脾脏形态大小正常，包膜完整光滑，内部回声分布均匀，脾门血管正常	
双肾	双肾形态大小位置正常，包膜完整，实质回声正常。双肾集合系统	

Ultrasound Findings (continued)

内见点状强回声伴彗星状声影，CDFI：肾内血流显示正常。

Preliminary impression:

- 初步意见
1. Mild fatty liver 1.轻度脂肪肝
 2. Bilateral renal crystals 2.双肾结晶

Prostate Ultrasound

检查者：陆洁

The prostate is normal in shape and size; the capsule is clear; internal echogenicity is homogeneous. No obvious abnormality detected.

前列腺

前列腺形态大小正常，包膜清晰，内部回声均匀

Thyroid Ultrasound

Both thyroid lobes are normal in shape and size; the surface is smooth; the capsule is intact; internal echogenicity is homogeneous. CDFI: normal blood flow signal.

Carotid Ultrasound

检查者：陆洁

The luminal diameter of both carotid arteries is normal. The intima-media is not thickened, the intima is smooth, and blood flow is normal.

甲状腺

双侧甲状腺形态大小正常，表面光滑，包膜完整，内部回声均匀。

Echocardiography

CDFI：血流显示正常。

Main measurements:

初步意见 未见明显异常

Aortic root diameter 28 mm; left atrial diameter 35 mm; interventricular septal thickness 10 mm; left ventricular posterior wall thickness 10 mm; left ventricular end-diastolic diameter 46 mm; left ventricular end-systolic diameter 27 mm; main pulmonary artery diameter 19 mm.

颈动脉彩超

Left ventricular ejection fraction: 71%; fractional shortening: 40%.

检查者：陆洁

Description: Cardiac chamber sizes are normal. No thickening of the left ventricular wall. Wall motion is normal. Valvular morphology, structure, and opening/closing are normal. Mitral inflow E wave > A wave. Spectral Doppler: no obvious abnormality. CDFI: no obvious abnormality.

初步意见

未见明显异常

心脏彩超

检查者：吕化冰

检查项目	检查所见	单位
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心脏

主要测值：

①主动脉根部内径28mm ②左房内径35mm ③室间隔厚度10mm ④左室后壁厚度10mm ⑤左室舒末内径46mm ⑥左室收末内径27mm ⑦主肺动脉内径19mm

左心功能测定：

①左室射血分数71% ②小轴缩短率40%

描述：各房室腔大小正常，左心室壁无增厚，室壁运动正常，各瓣膜形态、结构、启闭如常。

二尖瓣流速曲线：E峰>A峰。

频谱多普勒检查：未见明显异常。

CDFI：未见明显异常。

Radiology

Chest X-ray: 初步意见 未见明显异常

Both sides of the thorax are symmetric without deformity. Transparency of both lung fields is normal, and no abnormal dense shadow is seen. Lung markings are clear without increased prominence or distortion. Both hila are not enlarged or dense, and margins are clear. Both diaphragmatic surfaces are smooth, and both costophrenic angles are sharp. The aortic arch is prominent. Cardiac silhouette is not enlarged. The mediastinum is central and not widened.

Cervical Spine X-ray:

The vertebral alignment of the cervical spine is normal and the physiological curvature is preserved. Marginal osteophyte formation / sharpening is present at the 3rd, 5th, 6th, and 7th cervical vertebral bodies, with increased bone density. No obvious abnormality is seen in the remaining vertebral bodies or facet joints. Cervical intervertebral spaces are preserved. Calcification of the anterior longitudinal ligament is noted.

Impression:

- 1. Prominent aortic arch
- 2. Cervical osteophyte formation / cervical degenerative hyperostosis

Bone Density

Bone mass is normal. No obvious abnormality detected.

Stool Routine / Stool Occult Blood

These items were declined by consent and are therefore not included in this report.

初步意见 未见明显异常

便常规

此项目您已同意放弃检查，本次报告将不包含此检查结果。

便隐血

此项目您已同意放弃检查，本次报告将不包含此检查结果。